

THE NUTRITION CLUB CLINIC

Fatty Liver Guide

Nutrition and lifestyle basics for supporting liver health day to day.

Your plan should be shaped around your own test results and medical care.

What's inside

Fatty liver disease — now often called MASLD, metabolic dysfunction-associated steatotic liver disease — means excess fat has built up inside liver cells. It is extremely common, usually silent, and importantly it is one of the most reversible conditions in nutrition. This guide explains what drives it and what genuinely helps.

1. Understanding fatty liver
2. Signs, tests and what your results mean
3. The nutrition plan that works
4. Movement, sleep and habits
5. A practical week and what to track

How to use this guide

Focus on the fundamentals in sections 3 and 4 before anything else. Gradual weight loss, cutting liquid sugar and building muscle move liver fat far more than any supplement. Recheck blood tests with your doctor after about three to six months of consistent change.

1. Understanding fatty liver

A healthy liver contains very little fat. When more than roughly 5% of its weight is fat, it is called steatosis, or fatty liver. The liver is your metabolic hub: it stores and releases glucose, processes fats, makes proteins, and clears toxins, hormones and drugs.

The stages

- 1 **Simple steatosis** — fat accumulation without significant inflammation. Usually no symptoms.
- 2 **Steatohepatitis (MASH/NASH)** — fat plus inflammation and liver cell injury.
- 3 **Fibrosis** — scar tissue forms as the liver repairs repeated injury.
- 4 **Cirrhosis** — extensive scarring that impairs liver function; a small number progress to liver failure or cancer.

The good news: the earlier stages are largely reversible, and even fibrosis can improve with sustained metabolic change.

Who is at higher risk

- Excess weight, particularly around the waist — though lean people can have it too.
- Type 2 diabetes, prediabetes and insulin resistance.
- High triglycerides or low HDL cholesterol; high blood pressure.
- PCOS, hypothyroidism, obstructive sleep apnoea.
- High intake of sugary drinks, fructose and ultra-processed food; rapid weight gain.

Alcohol-related liver disease is different

MASLD is diagnosed when significant alcohol intake is excluded. If alcohol is a factor, the single most important step is reducing or stopping it — ask your doctor for support rather than going it alone.

2. Signs, tests and what your results mean

Most people have no symptoms and are diagnosed incidentally. When symptoms do appear, they are vague: tiredness, a dull ache or fullness in the upper right abdomen, and low energy.

Test	What it looks at	Notes
ALT and AST	Liver enzymes released when cells are injured	Can be normal even with significant fat; a raised ALT is a common first clue
GGT, ALP, bilirubin	Bile ducts and overall liver function	Help distinguish other causes
Ultrasound	Fat visible in the liver	The usual first imaging test; does not measure scarring well
FIB-4 or NAFLD fibrosis score	Calculated risk of scarring	Uses age, platelets and enzymes — free and useful for triage
FibroScan (elastography)	Liver stiffness and fat	Non-invasive way to estimate fibrosis stage

Your doctor will usually also check glucose or HbA1c, a lipid panel, thyroid function and rule out viral hepatitis and other causes.

Ask about your fibrosis risk

Fat alone matters less than scarring. Asking 'what is my FIB-4 score?' tells you whether you need specialist follow-up or can focus on lifestyle with routine monitoring.

3. The nutrition plan that works

1. Gradual, sustained weight loss (if you carry excess weight)

This is the best-evidenced treatment. Losing around **5%** of body weight reduces liver fat; **7–10%** can improve inflammation and even fibrosis. Aim for 0.5–1 kg per week — crash dieting can worsen liver inflammation.

2. Cut liquid sugar and refined fructose

The liver converts excess fructose directly into fat. Sugary drinks, fruit juice, energy drinks, sweetened tea and syrups are the highest-yield thing to remove. Whole fruit is fine — the fibre and water change everything.

3. Follow a Mediterranean-style pattern

It has the strongest evidence base for liver fat, independent of weight loss.

Eat more	Eat less
Vegetables and salad at both main meals; whole fruit; olive oil as the main fat; nuts and seeds; pulses; whole grains; oily fish 2–3 times a week; coffee (unsweetened); yoghurt	Sugary drinks and juice; white bread, pastries and biscuits; deep-fried and fast food; processed meat; alcohol; large portions of red meat; snacks made with refined oils and sugar

4. Enough protein

Roughly 1.2–1.6 g per kg of body weight daily protects muscle during weight loss and keeps you full. Spread it across meals: eggs, fish, poultry, pulses, dairy, tofu.

5. Coffee

Regular coffee (2–3 cups, without sugar and syrups) is consistently associated with lower liver fibrosis risk. Not a reason to start if you don't drink it, but no reason to stop.

About supplements

Vitamin E and omega-3 have some evidence in specific groups but are prescribed decisions, not self-service. Avoid high-dose iron unless deficient, and be very cautious with herbal 'liver detox' products — several are documented causes of liver injury. Your liver already detoxifies; it needs less burden, not more pills.

4. Movement, sleep and habits

Exercise reduces liver fat even without weight loss, so it is worth doing regardless of what the scale says.

- **Aerobic:** 150–240 minutes a week of brisk walking, cycling or swimming, spread over most days.
- **Resistance:** two or three sessions a week. Muscle absorbs glucose and improves insulin sensitivity.
- **Post-meal walks:** 10–15 minutes after eating lowers the glucose and insulin surge that drives fat storage.
- **Sit less:** long uninterrupted sitting is independently linked with liver fat.

Sleep and sleep apnoea

Short or fragmented sleep worsens insulin resistance. Loud snoring with daytime sleepiness may signal sleep apnoea, which is strongly linked with fatty liver — worth testing for.

Smoking and alcohol

Both accelerate fibrosis. Reducing alcohol to a minimum — or none if you have significant fibrosis — is one of the clearest protective steps available.

Medicines

Some drugs affect the liver, and your doctor may adjust treatment for diabetes, cholesterol or weight. Statins are generally safe and often beneficial in fatty liver despite common worries. Never stop prescribed medicine on your own.

5. A practical week and what to track

Sample day

- **Breakfast:** Oats made with milk, walnuts, cinnamon and berries; black coffee.
- **Lunch:** Large salad with olive oil, chickpeas or grilled chicken, whole grain bread.
- **Snack:** Plain yoghurt with a handful of nuts, or an apple.
- **Dinner:** Baked fish, roasted vegetables, small portion of brown rice or potato.
- **Drinks:** Water, unsweetened tea and coffee. No sugary drinks.

First four weeks — do these three things

- 1 Remove every sugar-sweetened drink and juice; replace with water or unsweetened tea.
- 2 Get vegetables or salad onto both main meals, and protein into breakfast.
- 3 Walk 30 minutes most days, including 10 minutes after your largest meal.

What to measure

Marker	How often	Why
Weight and waist circumference	Weekly, same conditions	Waist reflects visceral fat better than weight alone
ALT / AST	Every 3–6 months with your doctor	Often the first marker to improve
HbA1c and lipids	Every 6–12 months	Fatty liver travels with metabolic risk
Energy, sleep, waistband fit	Ongoing	Real-life feedback when numbers move slowly

Expect a slow, real improvement

Liver enzymes often start falling within 8–12 weeks of consistent change, and imaging can improve within 6–12 months. Consistency beats intensity here — the liver responds to the average of your year, not your best week.

Important note

This guide is general nutrition education only. It is not medical advice, a diagnosis, or a treatment plan, and it cannot account for your personal history, laboratory results, allergies or medicines. Always speak to your doctor, dietitian or pharmacist before making significant changes to your diet, supplements or activity — especially if you are pregnant or breastfeeding, have kidney, liver or heart disease, an eating disorder, or take prescription medication. If you experience severe or sudden symptoms, seek urgent medical care. Some herbal products and high-dose supplements are toxic to the liver. Always clear any supplement with your doctor when you have a liver condition.

Questions or want a personalised plan?

This guide covers general principles. A one-to-one consultation with The Nutrition Club Clinic lets us tailor portions, meal timing, food preferences, budget and any medications or conditions to you.

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